This paper estimates the "self-targeting" properties of the main means-tested transfer programs in the US. The authors find that program take-up is negatively correlated with consumption (a measure of permanent income) even controlling for current income (to control for causal effects of the program itself) and eligibility (to control for tagging), suggesting advantageous selection into means-tested programs. They develop a sufficient statistics framework to determine whether a marginal shift from voluntary to automatic transfers is desirable given the tradeoff between self-targeting and ordeals.

This is a very thought-provoking paper. It uses a simple and transparent methodology—comparing the characteristics (mainly consumption) of recipients and non-recipients conditional on eligibility—to show advantageous selection into means-tested programs. Although my prior is that selection into these programs is advantageous, I was struck by the magnitude of the selection and the comparisons across programs from Figure 1. I found Figure 1 to be the most compelling part of the paper because it compares programs on their targeting properties both relative to each other and after controlling for tags (eligibility).

I have the following suggestions:

1. My main concern is about the welfare analysis and whether it is consistent with the framing of "Should transfers be automatic?" (page 10). I appreciate that the "marginal" nature of the proposed reforms in the welfare analysis allows the authors to ignore the effect of the ordeal change on the inframarginal recipient. But one of the main arguments in favor of automatic enrollment is a reduction in stigma and ordeals for inframarginal recipients (the other argument is that the ordeals are so costly that they keep out eligible, and possibly deserving, people). For this reason, I would argue that the welfare analysis is not answering the question "Should transfers be automatic?" (since such an analysis would have to consider reductions in ordeals and stigma for inframarginal recipients) but rather "Should the marginal dollar be spent on a universal transfer or a targeted transfer under existing program design?" I would encourage the authors to either address what I would consider to be the "real" automatic enrollment proposal—a non-marginal change in ordeals—or change the framing so as not to overclaim. I think it is likely that targeted transfers are better than universal transfers even taking into account ordeals for the inframarginal recipient, but right now the deck is stacked against "automatic enrollment" as its proponents envision it. I think this is misleading and detracts from the contribution of the paper.

2. The authors should address the question of "scale" more directly. By the measure of self-targeting proposed in equation (3), we could find the one person with the lowest consumption level, make sure the program is designed such that only that one person applies and receives benefits, and declare a self-targeting victory because the average consumption of recipients is much lower than the average consumption of non-recipients. But by any sensible measure, this is a terrible program because there are plenty of "deserving" (low consumption) individuals who don't receive benefits. I don't see this point discussed at all in the empirical analysis. The tradeoff is implicit in the welfare analysis through the M parameter, but as far as I can see it is not directly discussed. Scale seems like a first-order parameter in estimating the welfare value of self-targeting.

3. The authors study self-targeting by looking at current consumption and lifetime income. These are useful measures of targeting under many models, but I would like to see the authors to specify what model(s) they have in mind and what alternative models they might consider. In particular, I was struck by the result that SSI is not as strong on self-targeting as other programs. This result is surprising for two reasons: (a) SSI has arguably the highest ordeals of any program considered, yet the authors find that it has the worst targeting on consumption; and (b) SSI targets people with disabilities, for whom consumption might map into the marginal utility of consumption differently than it does for able-bodied people (i.e., state-dependent utility).
a. With respect to point (a), if ordeals are very high for SSI but self-targeting is the worst of any program, does that mean that SSI has exceeded the "optimal" ordeal level? It seems like automatic enrollment for this kind of program could be cost-effective since the self-targeting properties are weak and the ordeals are high. In contrast, SNAP has the best self-targeting but arguably one of the lowest ordeal levels of any of these programs. It would be useful for the authors to engage more with the reasons for this heterogeneity across programs and discuss the extent to which self-targeting properties correlated with ordeal level.
b. With respect to point (b), it would be useful to know what model of welfare the authors have in mind when using the income and consumption measures, and whether they have considered robustness to state-dependent models or other alternatives. For example, other models might consider the macroeconomic countercyclical effects of these programs (e.g., maybe a program with low self-targeting properties based on consumption are those that go to households with a high MPC during a recession and its targeting properties are better in an MPC sense).

4. With respect to Figure 3, I was hoping that the authors would use the opportunity to consider the argument (sometimes labeled "behavioral," e.g., Bertrand Mullainathan and Shafir 2004, but also compatible with a neoclassical model) that there are people at the bottom of the consumption distribution who are screened out of these programs because they can't handle the application process. Is something like that happening for school meals or WIC? Should we be surprised that we don't see more of this in Figure 3?

Smaller points:
5. I am skeptical of the authors' ability to measure eligibility for certain programs. For some of these programs, examiners see much more detailed information than is available in the PSID. I appreciate that the authors acknowledge this point. In addition, it would be useful to detail the eligibility criteria of each program and rank them by the authors' confidence in measuring eligibility criteria.
6. Related to eligibility, I was glad to see that the authors consider an automatic counterfactual conditioning on eligibility. I would have thought this would be the main exercise instead of an appendix exercise, since my prior would be that ignoring tags would create first-order bias in the measurement of self-targeting.
7. I think the authors could better explain the relationship of their paper to prior research that takes similar approaches. On the empirical side, Lieber and Lockwood (2019) does essentially an identical exercise to evaluate the self-targeting properties of Medicaid home care. (The paper is referenced but only as an example of the targeting properties of ordeals, when in fact the targeting analysis in the paper is about Medicaid home care as a whole and not specifically about the ordeal.) Deshpande and Lockwood (2022) do an analysis that is similar in spirit for US

disability programs. On the theory side, Kleven and Kopczuk (2011) have a similar conceptual framework and similar results on the optimal design of means-tested transfers.